

Menopause and Cardiovascular Health

How does perimenopause and menopause affect cardiovascular health?

Perimenopause is the time in a woman's life when she is approaching menopause. During this time, her hormone levels (oestrogen and progesterone) are fluctuating and reducing over time. This leads to changes in menstrual periods until a woman stops seeing her menses in one year. At this time, the woman is said to be in menopause.

Menopause can occur naturally. But, it can also be caused from surgical removal of the ovaries or when the ovaries stop working well and are unable to control the menstrual cycle.

During this perimenopause or menopause, women experience hot flashes, sleep problems, bone changes and other problems that increase their risk of developing cardiovascular diseases. These problems include high blood pressure, high blood sugar, high blood cholesterol levels.

Therefore, perimenopause is an important time for women to evaluate their cardiovascular risk factors, and take steps to reduce them.

Does age at menopause affect cardiovascular risk?

Menopause in women who are less than 40 years is called premature menopause while menopause in women who are less than 45 years is called early menopause.

Premature or early menopause makes a woman to lose the protection that oestrogen offers to the heart at a young age. This increases the risk of developing heart diseases.

What effect does Hormone Replacement Therapy for menopause have on cardiovascular health?

The age of the woman, timing, route of administration and nature of the hormone replacement therapy affects the benefits or harms associated with use of hormone replacement therapy for managing cardiovascular risk in women with menopause.

Hormone replacement therapy when used in women above 60 years and more than 10 years since menopause, have been observed to have no benefits and may even increase the risk of developing cardiovascular diseases like stroke, venous thromboembolism, and heart attacks.

Whereas, hormone replacement therapy in women less than 60 years and less than 10 years since menopause have been observed to improve cardiovascular risk factors.

Transdermal hormone replacement therapy has better cardiovascular benefits when compared to pills.

Also use of formulations that are very identical to the hormones produced by the body are safer in reducing cardiovascular risks compared to the synthetic ones that do not resemble the hormones produced by the body.

Should a woman with menopause use hormone replacement therapy?

It is only a medical doctor that can evaluate the cardiovascular risk factor of a woman, consider her clinical history and determine if she needs hormone replacement therapy. It is dangerous to take hormone replacement therapy without the prescription of a doctor